

Outpatient Diabetes Cheat Sheet

UCSF endocrine modules (Rushakoff, Rao) · ADA 2022–23 era, verify drug details · Pre-op med/insulin dosing tool: [ucsf.logicnets.com](https://www.ucsflogicnets.com)

RULES

- **Choose by comorbidity, regardless of A1c:** HF/CKD → SGLT2i · ASCVD → GLP-1 RA or SGLT2i · weight → semaglutide/tirzepatide
- **Intensify if A1c >7.** Reassess q3–6 mo (avoid inertia)
- **Checkpoint-inhibitor DM → insulin now** · thin adult failing orals → **GAD65** (LADA)

Diagnosis

FPG ≥126 · A1c ≥6.5% · 2 h OGTT ≥200 · random ≥200 + sx (×2 unless unequivocal). A1c unreliable: hemoglobinopathy, pregnancy, HD, transfusion/bleed, EPO, dapsone → use glucose

Picking the drug

Situation	1st	Add
HF (HFrEF or HFpEF)	SGLT2i	GLP-1 RA
CKD (eGFR <60 or UACR ≥30)	SGLT2i (eGFR ≥20) + max ACEi/ARB	GLP-1 RA; finerenone if no SGLT2i
ASCVD / high risk	GLP-1 RA or SGLT2i	The other
Obesity	Semaglutide · tirzepatide	Avoid SU, TZD, insulin
Cost	Metformin → SU / pioglitazone	NPH / human insulin

Drug cheat sheet

Drug	Start → max	Renal	Watch
Metformin	500 bid → 1000 bid over 3 wk (w/ meals); ER if GI	eGFR 30–45 max 500 bid · <30 stop	GI · B12 yearly · no contrast hold
Semaglutide SQ	0.25 → 0.5 → 1 → 2 mg weekly		N/V → hold if vomiting (AKI)
Semaglutide PO	3 → 7 → 14 mg daily, 30 min before food		Same
Tirzepatide	2.5 mg weekly, ↑ 2.5 q4 wk → 15		Most wt loss · GI
Dulaglutide · liraglutide	0.75→4.5 mg weekly · 0.6→1.8 mg daily		GI
Empa · dapa · cana	10–25 · 5–10 · 100–300 mg daily	Benefit to eGFR ≥20	Yeast/UTI, volume, euDKA ; stop 3 d pre-op ; early eGFR dip OK
Glipizide · glimepiride	10–20 mg/d (XL 5–10) · 2–4 mg daily	↑ hypo in CKD	Hypo, weight; stop when prandial starts
Pioglitazone	15–45 mg daily		Edema, HF, fractures; 6–8 wk onset
Sitagliptin · linagliptin	100 mg (50/25 by CrCl) · 5 mg	Lina no adjust	Weak; not with GLP-1 RA

Insulin

- **Start basal:** glargine **10 U or 0.1–0.2 U/kg HS** (NPH at HS)
- **Titrate:** +2 U q3 d to FBG goal · unexplained hypo → ↓10–20%
- **Overbasalized** if basal >0.5 U/kg or big HS→AM drop → add GLP-1 RA or prandial, not more basal
- **Add prandial** (A1c >7 or AM/pre-dinner >140): 4 U or 10% of basal at largest meal; ↑ 1–2 U twice weekly. Stop SU
- **HS NPH** → **bid NPH:** 80% total, ⅔ AM / ⅓ HS · **Premix:** 0.3–0.5 U/kg, ⅔ AM / ⅓ PM
- **Very symptomatic new DM, no DKA:** insulin, or stop sugary drinks + max-dose SU and recheck 1 wk

Patient self-titration (for AVS): weekly, if FBG >120 on ≥2 d and no BG <80	120–140	141–160	161–180	>180
Increase basal by	+2 U	+4 U	+6 U	+8 U

Annual care

	Plan
Kidney	UACR + eGFR yearly · ACEi/ARB if UACR ≥30 + HTN (never both) · accept Cr ↑ ≤30% · + SGLT2i
Eye	Dilated exam yearly (q2 yr if normal + controlled)
Feet / nerve	Monofilament + vibration yearly · inspect feet each visit · B12 if on metformin Painful DPN: duloxetine 60–120, pregabalin, gabapentin
BP	< 130/80 · ACEi/ARB (1st if albuminuria/CAD), thiazide, DHP CCB · ≥160/100 → 2 drugs
Statin	Age 40–75: moderate · higher risk: high (LDL <70) · ASCVD: high (LDL <55) ± ezetimibe/PCSK9i
Aspirin	Secondary prevention. Primary only if ≥50 + risk factor + low bleed risk